

A scheduled Cesarean

Around one-third of Cesareans are scheduled, and the number of these has increased greatly in recent years. Reasons for a scheduled Cesarean include a breech baby; a tear involving the back passage, or another traumatic event during a previous vaginal delivery; a previous Cesarean; a larger than average baby; and maternal choice. In these situations, although a Cesarean is reasonable, an attempt at a vaginal birth is also reasonable with the right precautions.

In some situations, it would be hazardous for you or your baby if you were to try for a vaginal birth—for example, if you've had multiple previous Cesareans or other major surgery to the uterus; the baby can't be moved from lying horizontally; the placenta is lying very low (see [Low-lying placenta](#)); or there are severe pelvic abnormalities in the birth canal. Some maternal medical conditions also mean that vaginal birth is not advisable.

An elective Cesarean

Sometimes there may not be a medical reason that your obstetrician recommends a Cesarean for you. It is unusual for maternal request to be the only reason for a Cesarean, but it is also a valid choice if you have strong feelings against having a vaginal delivery. In this situation, your obstetrician may discuss your concerns over several prenatal visits, to make sure that you are certain an elective Cesarean is the right choice for you. Or you may start the labor process intending to have a vaginal birth and realize partway through that your gut instinct is telling you something different—take time to listen to your maternal instincts and discuss them with your obstetrician.

Timing of a Cesarean section

Most labor units will schedule Cesarean sections at set times during the weekdays to allow for adequate staffing and to maintain safety of all patients. Usually, a scheduled Cesarean is performed at, or just after, 39 weeks to reduce the risk of the baby developing breathing problems that may happen without the normal process of labor. In the uterus, the baby's lungs are full of fluid, and at birth, when the first breath is taken, all the fluid is absorbed into the bloodstream and the baby's lungs fill with air. The process of labor seems to help with this transition.

When a baby is born by scheduled Cesarean, there is no opportunity for them to prepare for the birth. At birth, the baby will take a first breath but a small amount of fluid may be left in the lungs making gas exchange more difficult. The baby must work harder to breathe and may make a grunting sound. This condition, known as transient tachypnoea of the newborn, is self-limiting. Often the baby only needs extra oxygen but, very occasionally, may need help with breathing. The earlier the baby is born, the higher the chance of them having breathing problems. However, after 39 weeks, the risk is very small.

If you go into labor before your scheduled Cesarean, contact your prenatal provider so you can be assessed immediately. If you are in active labor and do not want to do a VBAC, your doctor will get you ready for a [Cesarean](#) urgently.

VBAC

VBAC stands for Vaginal Birth After Cesarean. About 70 percent of women who go into labor having had a previous Cesarean will have a vaginal birth. And if they have also had a previous vaginal birth, the chance is increased to 90 percent. There is a very small risk, up to 1 percent, that during labor the previous Cesarean scar will open, known as uterine dehiscence, or uterine rupture. This is an emergency, requiring immediate delivery of the baby (within 15 minutes) as